

# TRT-GUY

## THE ENHANCED MALE FERTILITY RESTORATION GUIDE

**THE HCG + HMG PROTOCOL I USED TO GO FROM ZERO SPERM TO CONCEPTION**

Personal experience, protocol schedule, nutrition priorities, testing checkpoints, and practical FAQs.

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**IMPORTANT DISCLAIMER**

I am not a doctor, and this is not medical advice. This guide explains my personal experience and is for educational purposes only.

01

# HOW I RESTORED MY FERTILITY

## My experience

After years of PED use, my sperm count was zero.

I used a fertility-restoration protocol very similar to the one in this guide, brought my sperm back, and my wife and I conceived our first daughter without having to come off steroids (I was using higher doses back then).

This worked for me, and I want men in a similar position to understand the approach instead of automatically assuming their fertility is permanently gone.

## Before you start

- Get a formal semen analysis before starting. You need to know where things are at, and this will allow you to compare and decide if the protocol is working.
- Get morning testosterone, estradiol, LH, and FSH so you understand the hormonal picture.
- Write down every hormone, PED, SARM, peptide, medication, and supplement you use.

**THE TWO MANDATORY SEMEN ANALYSES**

Test once before starting and again at the 12-week mark. An optional week-8 semen analysis can show whether sperm has started appearing, but a zero result that early does not automatically mean the protocol has failed.

## 02

# THE HCG + HMG PROTOCOL

This schedule follows the incremental hCG + hMG protocol used by Finkel, Phillips, and Snyder. All gonadotropin doses below are measured in IU, not milligrams.

| Timeline           | hCG                                         | hMG                                          | Checkpoint                                                                                                      |
|--------------------|---------------------------------------------|----------------------------------------------|-----------------------------------------------------------------------------------------------------------------|
| Before starting    | None                                        | None                                         | Semen analysis.                                                                                                 |
| Weeks 1-16         | 2,000 IU Monday, Wednesday, Friday          | 75 IU Monday, Wednesday, Friday              | The drugs may be administered on the same days as separate injections. You can also alternate days if you wish. |
| Week 8             | Continue 2,000 IU three times per week      | Continue 75 IU three times per week          | Optional early semen analysis. A zero result this early does not prove failure.                                 |
| Week 12            | Continue 2,000 IU three times per week      | Continue 75 IU three times per week          | Primary semen analysis: concentration, total count, motility, morphology, and volume.                           |
| Weeks 17-32        | Continue 2,000 IU Monday, Wednesday, Friday | Increase to 150 IU Monday, Wednesday, Friday | Continue semen testing approximately every 8-12 weeks.                                                          |
| When sperm returns | Review with clinician                       | Review with clinician                        | Keep trying naturally and consider sperm banking while numbers are favorable.                                   |

## SAME DAYS OR ALTERNATING DAYS?

There is no established fertility advantage to alternating hCG on Monday, Wednesday, and Friday and hMG on Tuesday, Thursday, and Saturday. Using both on Monday, Wednesday, and Friday reduces the number of injection days and is commonly done. Keep them in separate syringes unless the prescriber or dispensing pharmacy specifically confirms they may be mixed.

## What each drug is doing

- hCG acts like LH. Its job is to stimulate the testes to produce testosterone locally.
- hMG provides FSH activity. Its job is to stimulate the Sertoli-cell side of sperm production.
- Using both targets the two hormonal signals that are normally suppressed by long-term testosterone or PED use.

## The study behind the incremental schedule

Finkel, Phillips, and Snyder used hCG 2,000 IU three times weekly, hMG 75 IU three times weekly for four months, and then hMG 150 IU three times weekly for another four months in men with hypogonadotropic hypogonadism. This is the exact published dosing schedule used in this guide.

03

# DO NOT CRASH YOURSELF WHILE TRYING TO CONCEIVE

## TRT, PEDs, and sexual function

You need sperm to conceive, but you also need libido, erections, energy, and a healthy relationship. In my opinion, deliberately crashing your testosterone and feeling terrible is not the ideal environment for trying to make a baby.

### MY PRACTICAL POSITION

Do not make yourself hypogonadal just because you are trying to conceive. Keep the androgen side as simple as possible and measure whether sperm is actually returning.

04

# EAT IN A SLIGHT CALORIE SURPLUS

This is not the time for an aggressive fat-loss phase. Sperm production is a resource-intensive biological process. I would rather give the body slightly more energy than it needs than ask it to restore fertility while dieting hard.

| Priority      | What I would do                                                                                               |
|---------------|---------------------------------------------------------------------------------------------------------------|
| Calories      | Stay in a slight calorie surplus. The goal is enough energy for recovery without rapid, unnecessary fat gain. |
| Protein       | Eat sufficient lean protein sources across the day to support recovery and general health.                    |
| Carbohydrates | Eat enough carbohydrates to support training, thyroid function, energy, and sexual performance.               |
| Produce       | Eat plenty of fruits and vegetables for micronutrients, fiber, and overall diet quality.                      |
| Fats          | Include sufficient dietary fat from quality sources. Do not run an unnecessarily low-fat diet.                |
| Food quality  | Build most meals from nutritious whole foods. Supplements should add to the diet, not replace it.             |

## 05

# INCREASE YOUR CHANCES OF CONCEPTION

## Have sex often

A protocol can improve the semen analysis, but conception still requires frequent intercourse. My practical target is sex every day while trying to conceive. During the fertile window, once or twice per day is an option if both partners feel good and it does not turn the process into a chore.

Use ovulation strips or another reliable method to identify the fertile window. The highest-priority days are the two days before ovulation and the day of ovulation.

## Optional additions

| Addition               | How I would use it                                                            | Purpose                                                                                                                                                       |
|------------------------|-------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Tadalafil              | 5 mg daily, prescribed by a physician                                         | More reliable erections and less performance pressure when intercourse needs to be frequent. This supports erectile function; it is not a direct libido drug. |
| Injectable L-carnitine | 200-500 mg three times per week; dose should follow the product and clinician | An optional addition intended to support sperm quality, cellular energy, and recovery.                                                                        |

## Other basics that still matter

- Sleep enough to recover and maintain sexual function.
- Avoid smoking, recreational drugs, excessive alcohol, and frequent high-heat exposure to the testes.
- Keep moderate training in, but do not combine hard dieting, excessive cardio, poor sleep, and maximal training volume.
- If erections, ejaculation, or desire are limiting intercourse, address the cause instead of pretending the problem is only motivation.

## 06

# MEASURE THE OUTCOME AND MAKE A DECISION

| Checkpoint                                      | Action                                                                                                                                       |
|-------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------|
| Before starting                                 | Formal semen analysis plus reproductive hormones.                                                                                            |
| Week 12                                         | Repeat the semen analysis. Compare concentration, total count, motility, morphology, and volume with baseline.                               |
| If sperm returns                                | Continue trying, discuss whether the protocol needs adjustment, and consider freezing sperm.                                                 |
| If sperm is still zero after 5 months           | Confirm the diagnosis and discuss whether more time, a change in stimulation, or a fertility procedure is appropriate. Contact a specialist. |
| If semen improves but conception does not occur | Have your partner evaluated. Fertility is a couple outcome, and the male side is only one part of the equation.                              |

## MY BOTTOM LINE

I went from zero sperm to conceiving my first daughter. I believe men should know that fertility can sometimes be restored after long-term PED use. Get the data, run a consistent plan, fuel the body, have sex often, and measure the result.

07

# FREQUENTLY ASKED QUESTIONS

## Do I need to come off TRT for this to work?

No.

## Do I need to come off a steroid cycle?

No.

## Can I guarantee that I will regain fertility?

No. I regained fertility and conceived my first daughter, but that is my personal outcome. The result depends on your fertility before PED use, age, duration of suppression, testicular function, genetics, medication quality, adherence, and your partner's fertility.

## How long does it take for sperm to return?

Sperm production takes roughly 74 days, followed by additional maturation and transport. Some men show sperm by 8-12 weeks, while others need 4-6 months or longer. A zero result at three months does not automatically mean permanent infertility.

## Should I test at week 8 or wait until week 12?

Week 8 is an optional early look. Week 12 is the more useful first assessment because it allows approximately one sperm-production cycle. Use the same laboratory and follow its abstinence instructions so the results are easier to compare.

## Is hCG alone enough?

Sometimes. hCG supplies LH-like stimulation and may be enough for men with preserved testicular function. hMG or recombinant FSH adds direct FSH activity and is often considered when hCG alone does not restore adequate sperm or when faster, stronger stimulation is needed.

## Does using hCG while on TRT guarantee that I stay fertile?

No. Men frequently assume that hCG automatically preserves fertility, but semen responses vary. The only way to know what is happening is to perform semen analyses. Test before TRT or PED use when possible, and do not rely on testicular size or ejaculate volume.

## Do I need Clomid, enclomiphene, or Nolvadex as well?

They were not part of my protocol, therefore I can't recommend them.

## What if hCG raises my estradiol or causes side effects?

hCG can raise testosterone and estradiol. Water retention, breast tenderness, acne, mood changes, headaches, or feeling overstimulated are reasons to review the dose and work with a coach or clinician.

## How often should we have sex?

My practical target is daily sex while trying to conceive, especially during the fertile window.

## When should my partner be evaluated?

Ideally, early rather than only after months of male treatment. If the female partner is 35 or older, has irregular cycles, or has a known reproductive issue, time matters even more. Fertility is a couple outcome.

## Should I freeze sperm when it comes back?

It is worth discussing. Sperm counts can fluctuate, medication access can change, and future TRT or PED use may suppress fertility again. Banking sperm when a usable sample appears can provide insurance for the future.

## Can I mix hCG and hMG in the same syringe?

They may be scheduled on the same days, but you might need them in separate syringes due to the volume of each injection.

## SUPPORTING SOURCES

Finkel DM, Phillips JL, Snyder PJ. New England Journal of Medicine. 1985;313:651-655. doi:10.1056/NEJM198509123131101.

Menon DK. Fertility and Sterility. 2003;79 Suppl 3:1659-1661. doi:10.1016/S0015-0282(03)00365-0.

AUA/ASRM. Diagnosis and Treatment of Infertility in Men. Guideline amended 2024.